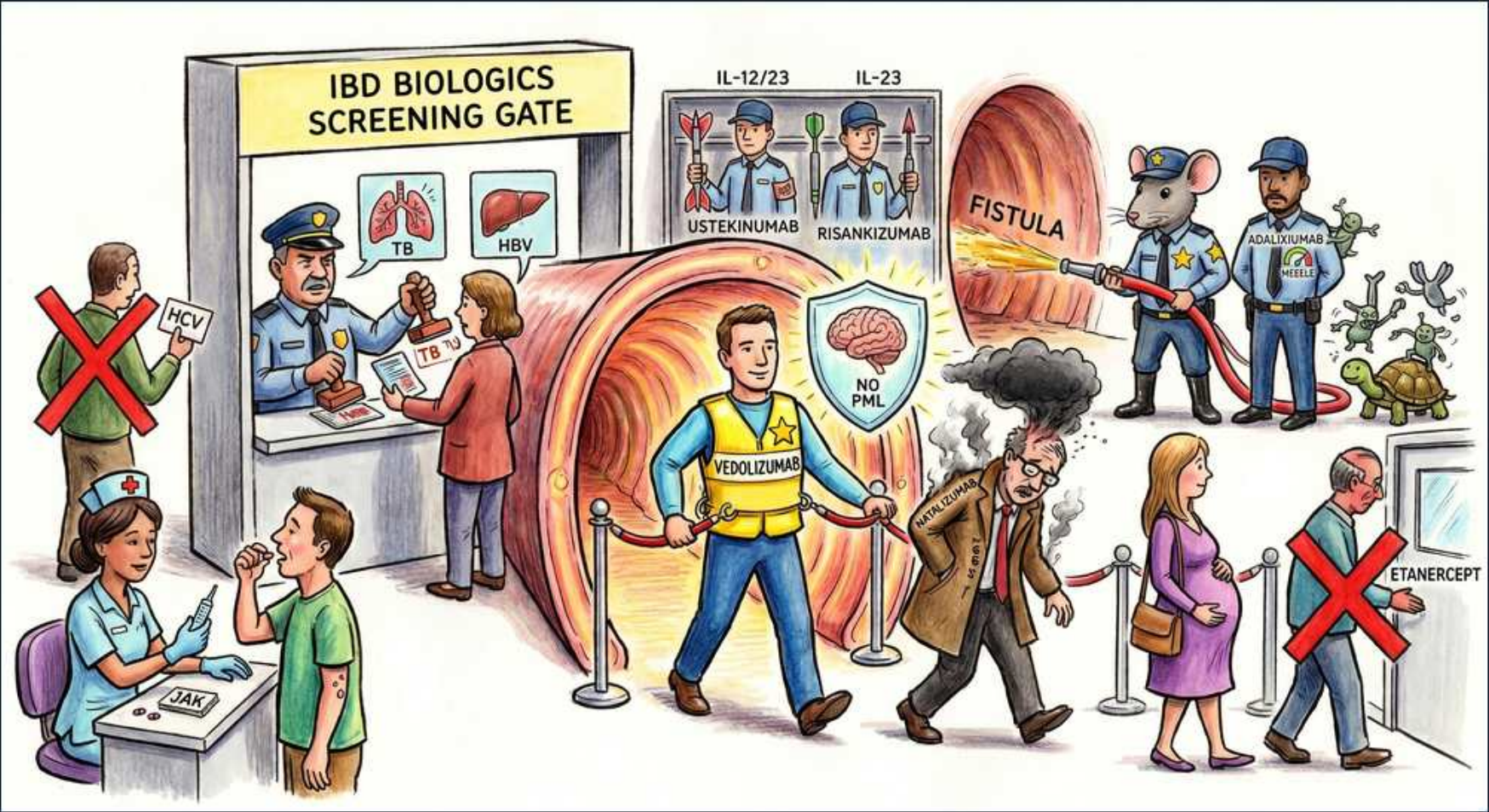


IBD — Biologics & Pre-Treatment Screening



1. Screening gate (TB + HBV)

WHAT YOU SEE

Booth 'IBD BIOLOGICS SCREENING GATE' with guards holding 'TB' (lungs) and 'HBV' cards.

WHAT IT ENCODES

Before starting biologics (especially anti-TNF) screen for latent TB (IGRA/PPD + CXR) and hepatitis B (HBsAg, anti-HBc) — both can reactivate. Treat latent TB before biologics; manage HBV with antiviral prophylaxis.

2. HCV crossed out (red X)

WHAT YOU SEE

Card 'HCV' on the left held up and crossed with a red X.

WHAT IT ENCODES

Routine HCV screening is NOT a required pre-biologic step in IBD (TB and HBV are the mandated screens). HCV does not reactivate with anti-TNF the way HBV does — a board trap.

BOARD TRAP

"Screen for HCV before biologics" → wrong; required pre-biologic screens are latent TB and HBV, not HCV.

3. Zoster/Shingrix before JAK (nurse)

WHAT YOU SEE

Nurse at a 'JAK' desk vaccinating a patient's arm.

WHAT IT ENCODES

Give the recombinant zoster (Shingrix) vaccine BEFORE starting a JAK inhibitor — JAK inhibitors markedly raise herpes zoster risk. JAKs also carry a black-box warning (thrombosis/VTE, MI, malignancy, serious infection).

4. Vedolizumab vest — NO PML (gold star)

WHAT YOU SEE

Central figure in safety vest 'VEDOLIZUMAB' holding a shield 'NO PML'.

WHAT IT ENCODES

Vedolizumab is gut-selective: anti- $\alpha 4\beta 7$ integrin blocking MAdCAM-1 binding, so it acts only on gut lymphocyte trafficking — NO PML risk (the shield). Slow onset; excellent safety profile; good for UC and Crohn's.

5. Natalizumab smoke man (red X)

WHAT YOU SEE

Man emitting black smoke labeled 'NATALIZUMAB', red X nearby.

WHAT IT ENCODES

Natalizumab is anti- $\alpha 4$ integrin (non-selective) and crosses into the CNS → JC-virus reactivation causing PML (progressive multifocal leukoencephalopathy). Rarely used in IBD because of this; check anti-JCV antibody status.

BOARD TRAP

"Natalizumab is gut-selective and PML-free" → wrong; it is anti- $\alpha 4$ (non-selective) and causes PML.

6. Infliximab fireman on fistula (gold star)

WHAT YOU SEE

Figure spraying a hose into a tunnel labeled 'FISTULA', card 'ADALIMUMAB' nearby on the mouse.

WHAT IT ENCODES

Infliximab = chimeric (mouse-human) IV anti-TNF, BEST agent for FISTULIZING Crohn's. The chimeric portion is more immunogenic. Gold star = first-line for fistulizing CD.

7. Adalimumab mouse (anti-drug Ab)

WHAT YOU SEE

A mouse character labeled 'ADALIMUMAB' near the fistula spray.

WHAT IT ENCODES

Adalimumab = fully human subcutaneous anti-TNF; can still form anti-drug antibodies, so combine with an immunomodulator (e.g., AZA/MTX) to reduce immunogenicity and loss of response. Self-injected, with a weekly dosing option.

8. Ustekinumab dart (IL-12/23)

WHAT YOU SEE

Figure throwing darts labeled 'USTEKINUMAB' under 'IL-12/23'.

WHAT IT ENCODES

Ustekinumab targets the shared p40 subunit of IL-12 and IL-23. Used in Crohn's and UC; favorable safety, good for anti-TNF failures.

9. Risankizumab dart (IL-23)

WHAT YOU SEE

Figure throwing darts labeled 'RISANKIZUMAB' under 'IL-23'.

WHAT IT ENCODES

Risankizumab selectively targets the p19 subunit of IL-23 — newer, effective for Crohn's. Selective IL-23 blockade is the trend beyond combined IL-12/23 inhibition.

10. Pregnant traveler passes safely

WHAT YOU SEE

Visibly pregnant woman walking through past the gates unobstructed.

WHAT IT ENCODES

Anti-TNF agents are SAFE in pregnancy and generally continued to maintain remission (active disease is the bigger fetal risk). IgG1 anti-TNF crosses the placenta mainly in the 3rd trimester; coordinate live-vaccine timing for the infant.

BOARD TRAP

"Anti-TNF is contraindicated in pregnancy" → wrong; anti-TNFs are considered safe and usually continued.

11. Etanercept blocked at door (red X)

WHAT YOU SEE

Far-right figure at a doorway crossed out, labeled 'ETANERCEPT'.

WHAT IT ENCODES

Etanercept (TNF-receptor decoy) is ineffective in IBD — joint disease only. The effective IBD anti-TNFs are the monoclonals infliximab and adalimumab.

BOARD TRAP

"Etanercept works for IBD" → wrong; it is ineffective in IBD; use infliximab or adalimumab.
